

COMMITTEE ON HEALTH AND HUMAN SERVICES
SENATE AMENDMENTS TO S.B. 1611
(Reference to printed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

~~[GREEN STRIKEOUT IN BRACKETS]~~ indicates new text removed from statute or previously enacted session law.

~~[Green strikeout in brackets]~~ indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

~~<<Green strikeout in carets>>~~ indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Title 36, chapter 29, article 1, Arizona Revised
3 Statutes, is amended by adding section 36-2903.18, to read:

4 36-2903.18. American Indian health program; administrative
5 services contract; legislative review;
6 definitions

7 A. BEGINNING OCTOBER 1, 2027, THE ADMINISTRATION SHALL CONTRACT
8 WITH A QUALIFIED CONTRACTOR OR OTHER QUALIFIED ENTITY TO SERVE AS THE
9 ADMINISTRATIVE SERVICES ORGANIZATION AND TO PERFORM ~~[PROGRAM INTEGRITY AND~~
10 ~~CARE MANAGEMENT]~~ ~~[THE]~~ FUNCTIONS ~~[PRESCRIBED IN SUBSECTION H OF THIS~~
11 ~~SECTION]~~ FOR MEMBERS ENROLLED IN THE AMERICAN INDIAN HEALTH PROGRAM~~[, NOT~~
12 ~~INCLUDING SERVICES PROVIDED TO AMERICAN INDIAN OR ALASKA NATIVE MEMBERS~~
13 ~~THROUGH THE INDIAN HEALTH SERVICE OR A TRIBAL FACILITY]~~. THE CONTRACT
14 MUST COMPLY WITH ALL APPLICABLE FEDERAL AND STATE REQUIREMENTS, INCLUDING
15 THE PROTECTIONS FOR INDIANS, INDIAN HEALTH CARE PROVIDERS AND INDIAN
16 MANAGED CARE ENTITIES PURSUANT TO 42 CODE OF FEDERAL REGULATIONS SECTION
17 438.14.

18 B. THE ADMINISTRATION SHALL RETAIN ULTIMATE RESPONSIBILITY FOR
19 ADMINISTERING THE AMERICAN INDIAN HEALTH PROGRAM, INCLUDING FEE-FOR-SERVICE
20 RATE SETTING~~[, CLAIMS PAYMENT AUTHORITY]~~ AND OVERSIGHT OF ANY
21 ADMINISTRATIVE SERVICES ORGANIZATION CONTRACTOR, AND MAY NOT ELIMINATE THE
22 AMERICAN INDIAN HEALTH PROGRAM AS A FEE-FOR-SERVICE OPTION FOR ELIGIBLE
23 AMERICAN INDIAN AND ALASKA NATIVE MEMBERS.

24 C. BEFORE ISSUING ANY REQUEST FOR PROPOSALS TO PROCURE AN
25 ADMINISTRATIVE SERVICES ORGANIZATION PURSUANT TO SUBSECTION A OF THIS
26 SECTION:

27 1. NOT LATER THAN THIRTY DAYS AFTER THE EFFECTIVE DATE OF THIS
28 SECTION, THE DIRECTOR SHALL REQUEST ANY NECESSARY APPROVALS FROM THE

1 CENTERS FOR MEDICARE AND MEDICAID SERVICES TO IMPLEMENT THIS SECTION AND
2 TO QUALIFY FOR FEDERAL MONIES AVAILABLE UNDER TITLE XIX OF THE SOCIAL
3 SECURITY ACT OR THE SECTION 1115 WAIVER.

4 2. NOTWITHSTANDING ANY LAW TO THE CONTRARY:

5 (a) THE ADMINISTRATION SHALL PRESENT THE PROPOSED PROCUREMENT
6 STRATEGY, MINIMUM QUALIFICATIONS AND EVALUATION FACTORS TO THE SENATE AND
7 HOUSE OF REPRESENTATIVES HEALTH AND HUMAN SERVICES COMMITTEES, OR THEIR
8 SUCCESSOR COMMITTEES.

9 (b) THE ADMINISTRATION SHALL RECEIVE AND [THOUGHTFULLY] CONSIDER
10 [AND MAY RESPOND TO] WRITTEN AND ORAL COMMENTS FROM COMMITTEE MEMBERS AND
11 THE PUBLIC, INCLUDING TRIBAL GOVERNMENTS, INDIAN HEALTH CARE PROVIDERS AND
12 URBAN INDIAN ORGANIZATIONS.

13 D. NOTWITHSTANDING ANY LAW TO THE CONTRARY, FOR EACH PROCUREMENT
14 DESCRIBED IN SUBSECTION C OF THIS SECTION:

15 1. THE [CHAIRPERSONS] [PRESIDENT] OF THE SENATE AND [THE SPEAKER OF
16 THE] HOUSE OF REPRESENTATIVES [HEALTH AND HUMAN SERVICES COMMITTEES, OR
17 THEIR SUCCESSOR COMMITTEES, MAY EACH DESIGNATE ONE MEMBER OF THEIR
18 RESPECTIVE COMMITTEE TO SERVE AS A] [SHALL EACH APPOINT ELEVEN] NONVOTING
19 [OBSERVER] [OBSERVERS WHO ARE MEMBERS OF DIFFERENT FEDERALLY RECOGNIZED
20 INDIAN TRIBES] TO THE SOURCE-SELECTION EVALUATION COMMITTEE CONVENED BY
21 THE ADMINISTRATION.

22 2. THE NONVOTING OBSERVERS DESIGNATED PURSUANT TO PARAGRAPH 1 OF
23 THIS SUBSECTION MAY ATTEND EVALUATION MEETINGS AND REVIEW MATERIALS
24 SUBMITTED BY OFFERORS BUT MAY NOT SCORE PROPOSALS, PARTICIPATE IN FINAL
25 RANKING OR MAKE OR DIRECT AN AWARD DECISION.

26 E. NOTWITHSTANDING ANY LAW TO THE CONTRARY, AFTER THE COMPLETION OF
27 PROPOSAL EVALUATIONS AND BEFORE THE EXECUTION OF ANY CONTRACT DESCRIBED IN
28 SUBSECTION A OF THIS SECTION, THE ADMINISTRATION SHALL TRANSMIT AND
29 PRESENT TO THE SENATE AND HOUSE OF REPRESENTATIVES HEALTH AND HUMAN
30 SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, A SUMMARY OF
31 EVALUATION RESULTS AND THE PROPOSED AWARD RECOMMENDATION.

32 F. THIS SECTION DOES NOT:

33 1. TRANSFER SOURCE-SELECTION OR CONTRACT-AWARD AUTHORITY FROM THE
34 ADMINISTRATION TO THE LEGISLATURE~~[.]~~ [OR] ANY LEGISLATIVE COMMITTEE [OR
35 ANY TRIBAL GOVERNMENT].

36 2. CONFLICT WITH THE ARIZONA PROCUREMENT CODE OR ANY APPLICABLE
37 FEDERAL PROCUREMENT REQUIREMENT. THE ADMINISTRATION REMAINS THE AWARDED
38 AGENCY AND SHALL COMPLY WITH ALL APPLICABLE STATE AND FEDERAL PROCUREMENT
39 LAWS AND REGULATIONS.

40 G. ANY ADMINISTRATIVE ARRANGEMENT AUTHORIZED UNDER THIS SECTION
41 MUST PRESERVE:

42 1. THE RIGHT OF ELIGIBLE AMERICAN INDIAN AND ALASKA NATIVE MEMBERS
43 TO ELECT THE AMERICAN INDIAN HEALTH PROGRAM FEE-FOR-SERVICE COVERAGE.

44 2. THE RIGHT OF AMERICAN INDIAN AND ALASKA NATIVE MEMBERS TO
45 VOLUNTARILY ENROLL IN AND DISENROLL FROM A MANAGED CARE ORGANIZATION
46 CONSISTENT WITH FEDERAL LAW.

1 3. THE PROTECTIONS AFFORDED TO INDIANS AND INDIAN HEALTH CARE
2 PROVIDERS UNDER 42 CODE OF FEDERAL REGULATIONS SECTION 438.14, INCLUDING
3 OUT-OF-NETWORK ACCESS AND APPROPRIATE PAYMENT REQUIREMENTS FOR INDIAN
4 HEALTH CARE PROVIDERS.

5 H. ANY ENTITY THAT IS PROCURED PURSUANT TO THIS SECTION TO PERFORM
6 PROGRAM INTEGRITY~~[.]~~ ~~[OR]~~ CARE MANAGEMENT~~[.] PROVIDER SUPPORT, QUALITY~~
7 ~~IMPROVEMENT AND DATA ANALYTICS]~~ FUNCTIONS ~~[AND CLAIMS PAYMENT]~~ FOR THE
8 AMERICAN INDIAN HEALTH PROGRAM SHALL BE ORGANIZED AND OPERATED AS A
9 QUALIFIED CONTRACTOR. ~~[THE ADMINISTRATIVE SERVICES ORGANIZATION'S~~
10 ~~PERFORMANCE OF PROGRAM INTEGRITY FUNCTIONS DOES NOT SUPERSEDE THE DUTIES~~
11 ~~AND RESPONSIBILITIES OF THE ADMINISTRATION'S OFFICE OF INSPECTOR GENERAL~~
12 ~~TO INVESTIGATE FRAUD.]~~

13 I. FOR THE PURPOSES OF THIS SECTION:

14 1. "ADMINISTRATIVE SERVICES ORGANIZATION" MEANS A MEDICAID MANAGED
15 CARE ORGANIZATION THAT IS A QUALIFIED CONTRACTOR OR OTHER QUALIFIED ENTITY
16 THAT CONTRACTS WITH THE ADMINISTRATION TO PERFORM ADMINISTRATIVE
17 FUNCTIONS, INCLUDING PROGRAM INTEGRITY, CARE MANAGEMENT, PROVIDER
18 ~~[NETWORK]~~ SUPPORT, QUALITY IMPROVEMENT~~[.]~~ ~~[AND]~~ DATA ANALYTICS ~~[AND CLAIMS~~
19 ~~PAYMENT]~~.

20 2. "AMERICAN INDIAN HEALTH PROGRAM"~~[:]~~

21 ~~(a)]~~ MEANS THE FEE-FOR-SERVICE PROGRAM OPERATED BY THE
22 ADMINISTRATION FOR ELIGIBLE AMERICAN INDIAN AND ALASKA NATIVE MEMBERS AS
23 REQUIRED UNDER FEDERAL LAW TO PRESERVE A FEE-FOR-SERVICE OPTION FOR
24 AMERICAN INDIAN AND ALASKA NATIVE BENEFICIARIES.

25 ~~[(b) DOES NOT INCLUDE SERVICES PROVIDED TO AMERICAN INDIAN OR~~
26 ~~ALASKA NATIVE MEMBERS THROUGH THE INDIAN HEALTH SERVICE OR A TRIBAL~~
27 ~~FACILITY.]~~

28 3. "CARE MANAGEMENT" MEANS THE COORDINATION OF A MEMBER'S HEALTH
29 CARE SERVICES TO ENSURE MEDICALLY NECESSARY AND APPROPRIATE CARE.

30 4. "DATA ANALYTICS" MEANS THE COLLECTION, INTEGRATION, ANALYSIS AND
31 REPORTING OF PROGRAM CLAIMS AND PROVIDER AND MEMBER DATA TO SUPPORT
32 PROGRAM OPERATIONS, DECISION-MAKING AND OVERSIGHT.

33 5. "PROGRAM INTEGRITY" MEANS FUNCTIONS DESIGNED TO PREVENT, DETECT
34 AND INVESTIGATE FRAUD, WASTE OR ABUSE AND TO RECOVER IMPROPER PAYMENTS IN
35 THE ADMINISTRATION OR DELIVERY OF COVERED SERVICES.

36 6. "PROVIDER SUPPORT" INCLUDES:

37 ~~(a) PROVIDER EDUCATION, TRAINING AND TECHNICAL ASSISTANCE.~~

38 ~~(b) ASSISTANCE WITH CLAIMS SUBMISSION AND BILLING ISSUES.~~

39 ~~(c) COMMUNICATION OF PROGRAM POLICIES, PROCEDURES AND UPDATES.]~~

40 ~~[5:]~~ ~~[7.]~~ "QUALIFIED CONTRACTOR" OR "OTHER QUALIFIED ENTITY" MEANS
41 AN ENTITY THAT MEETS ALL APPLICABLE FEDERAL AND STATE REQUIREMENTS TO
42 CONTRACT WITH THE ADMINISTRATION AS A MEDICAID MANAGED CARE ORGANIZATION,
43 ADMINISTRATIVE SERVICES ORGANIZATION OR SIMILAR ENTITY.

44 ~~[8. "QUALITY IMPROVEMENT" MEANS MEASURING, MONITORING AND IMPROVING~~
45 ~~THE QUALITY, SAFETY AND OUTCOMES OF HEALTH CARE SERVICES.]~~

1 Sec. 2. Legislative findings

2 The legislature finds and determines that immediate implementation
3 of this act is necessary to strengthen program integrity and care
4 management for American Indian and Alaska native members of the Arizona
5 health care cost containment system, to protect those members from fraud
6 and abuse and to preserve the public peace, health and safety.

7 Sec. 3. Emergency

8 This act is an emergency measure that is necessary to preserve the
9 public peace, health or safety and is operative immediately as provided by
10 law.

11 Enroll and engross to conform

12 Amend title to conform

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